

CASE MANAGER HANDOUT

PRE-DEMAND & REDUCTION FILE REVIEW PROTOCOLS

This operational handout maps out your standard operating procedures for auditing active case files across the Pre-Suit, Pending Demand, and Litigation phases. Your precision throughout this review preserves core case valuations and structurally prepares the file for aggressive medical bill reductions—maximizing your operational efficiency and protecting your client's final net recovery.

1. Initial System Setup & Representation Audit

Before digging into the clinical documents, you must align the file across platforms to verify exactly where the final demand needs to be routed.

Step 1: System Mapping

- **Action:** Open your case file simultaneously in Casepeer (CP) and Sharepoint (SP). Assign your name as the official 'Lien Negotiator' inside Casepeer on the master file and all secondary linked case profiles.

Step 2: Determine the Target Recipient

The path of your demand packet is completely dictated by the file's litigation status:

Active Litigation Files: On Sharepoint, navigate directly to the folder path: Litigation / Complaint and Summons / Answer to affirmative defenses and reply / Answer. If a defense Answer is present, extract opposing counsel's (OC) firm name and handling attorney details. Add this legal representation profile under the Defendant section on Casepeer and route your demand packet directly to them.

Pre-Suit / Unrepresented Defense Files: If no formal Answer is logged, address your demand letter directly to the handling third-party insurance adjuster.



Step 3: Verify Coverage Boundaries & Demand Logic Triggers

Cross-examine the insurance modules on Casepeer to confirm third-party Bodily Injury (BI) and first-party Underinsured/Uninsured Motorist (UM) policy limits. Apply these rules to map out your routing strategy:

Pending Demand Status Files: If both BI and UM coverages exist, you **MUST** send concurrent demands to both insurance carriers if: (1) The client's outstanding medical balances **EXCEED** the available third-party BI limits (clearly seen once your Net Sheet is built), or (2) The third-party BI limits are low or near-equal to the face value of the client's outstanding medical liabilities.

Litigation Status Files Only: If UM coverage is available but the client's total outstanding medical bills **DO NOT** exceed the primary BI coverage limits, do not generate a UM demand packet at all.

2. Sharepoint Document Checklist Matrix (Stage 1 Audit)

Initialize your master Net Sheet utilizing the most recent template version dated 2/3/24. Download the Demand Checklist directly from Casepeer (Defense-Plaintiff Ins. tab / Adjuster tab / Demand Checklist) and save a working copy in the client's 'Demand and Settlement / Demand Packet' folder.

Systematically audit the following Sharepoint folders against your Casepeer intake notes:

Folder Segment	Audit Evaluation Standard	Corrective Action Trigger
Intake Notes & ER	Cross-examine the intake description for transit details. If transported by emergency services or if an ER visit occurred post-impact, matching records and bills must be present. Verify signed retainer, client ID, and active health insurance cards.	Task your responsible Virtual Assistant (VA) to request missing elements from the client or facility.
Police Report (PR)	Check for scene treatment logs. Operational Rule: If the client was evaluated by EMS at the scene but WAS NOT transported, no EMS bill will be generated.	DO NOT assign follow-up tasks for EMS billing if no transport occurred.



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Folder Segment	Audit Evaluation Standard	Corrective Action Trigger
Insurance Disclosures	Confirm presence of complete disclosures. • Defense: Affidavit of Coverage, Executed Affidavit of Additional Insured, Dec Pages. • Plaintiff: Affidavit of Coverage, Dec Pages, UM Selection/Rejection Form.	If an Add. Insured Affidavit is missing after initialization, execute a maximum of 3 follow-ups, then formally close the task. Task appropriate staff for other missing dec items.
Property Damage (PD)	Confirm the presence of vehicle estimates, total loss evaluation sheets (if applicable), supplemental sheets, and scene photos. Verify the vehicle data matches your client or the owner of the vehicle they operated.	Task staff if missing. Operational Rule: If the total property damage estimate is under \$1,000, exclude it from the final demand packet entirely.
Prior Attorney Folder	Look for a signed termination letter. Check for fax/email confirmation receipts and formal proof of mailing to clear clean title.	If no signed termination letter exists, task the responsible VA to have the client sign one immediately.
Pre-Settlement Funding	Scan for advanced living or litigation funding records.	Formally task appropriate staff to pull an updated payoff letter and log the balance into Casepeer.
Dynamic Liens	Check secondary folders for alternative dynamic claims (e.g., Child Support Liens, Workers' Compensation, MedPay).	Log balances on the Net Sheet and task follow-ups accordingly.

3. Clinical Chronology & Reductions Audit (Stage 2)

As you compile your medical exhibits in strict chronological order, you must simultaneously input all clinical data (total charges, write-offs, balances, and specific date ranges) into both the Casepeer Treatment section and your Net Sheet. Use these specific diagnostic review rules:



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A. First-Party Insurance Tracking (PIP Log)

- **The 60-Day Rule:** Your PIP log must be less than 60 days old. If it is older, a new itemized log must be requested immediately. Verify that every medical provider paid on that log is created as an independent profile on Casepeer.
- **Litigation Exemption:** Do not hold back a litigation demand packet while waiting for an updated PIP log; keep tracking the document via tasks but ship the demand. Never include a copy of the PIP log in outgoing litigation demands.

B. Medical Provider Review Guidelines

- **EMS:** Confirm itemized ledger and run logs/trip sheets are intact. Operational Rule: EMS bills are never negotiated down; always project them at full face value on your Net Sheet.
- **Emergency Room (ER):** Secure complete emergency charting (do not accept summary discharge summaries). Locate the 'Chief Complaint' on the clinical report to verify that the reason for the visit is directly linked to the Motor Vehicle Accident (MVA). Look over ER billing ledgers for health insurance line-item payments. If health insurance cleared payments, log the provider under a Health Subrogation Lien on your Net Sheet and request a final statutory lien payoff statement. (Note: ER bills are not negotiated unless the client is completely upside down at final settlement).
- **Chiropractic / Physical Therapy:** Confirm presence of initial evaluation, full SOAP progress sheets, final discharge summary, and itemized ledger. Carefully check dates of service to ensure these records are tied to the current accident and not mixed up with a historic injury file. Read through the full provider chart; high-volume chiropractors frequently embed external references to sub-providers (e.g., secondary referrals for an EMC, diagnostic X-rays, or specialized MRI center runs) inside their own chart. You must pull matching independent billing ledgers for every sub-provider found.
- **Emergency Medical Condition (EMC) & Diagnostics:** EMC: Confirm the final diagnostic report and standalone billing statement are logged.
MRIs: Audit ledgers to confirm no scan views are missing and that no non-accident dates of service are listed. If an old date appears, task staff to have the provider remove the non-related line item before it reaches defense adjusters.
- **Orthopedic Specialists:** Check reports to verify the correct Date of Loss (DOL) is listed. Look for mentions of hidden medical history or ER visits the client forgot to disclose during intake. Cross-reference all Dates of Service (DOS) appearing on the ortho billing ledger against your medical reports to ensure your records are complete. Task staff for any missing records.



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- **Surgeries & Invasive Procedures:** Operational Rule: You CANNOT mark a file as demand-ready if surgical billing components are missing. For injections or invasive procedures, you must gather the entire sub-billing suite: the primary surgical facility ledger, separate anesthesia bills, medical assistant/hardware invoices, implant sheets, and post-operative brace charges. Refer to your firm master guidelines to confirm which procedures are classified as actual surgeries.

4. Proportional Reduction & Max Offer Matrix

To model realistic maximum reduction offer caps on your client's Net Sheet during your file review, apply these strict guideline matrices:

Chiropractic & Physical Therapy Caps

- **PIP Paid \$8,000+ (Near Policy Cap):** If a high balance remains, set the maximum top reduction offer at \$2,500. If the outstanding balance is low, limit the maximum top offer to \$1,500.
- **PIP Paid \$10,000 (Full Policy Cap):** Aim for an absolute waiver of the remaining balance. If the chiro refuses to waive it, enforce a strict maximum top offer of \$1,000.
- **Pagano Chiropractic Exception:** Always structure your initial negotiation offer at their maximum top limit (initial offer matches outstanding balance limits closely; do not lowball initially).
- **PIP Paid Under \$8,000:** The maximum top offer cap can range between \$3,000 and \$4,000, heavily dependent on the global settlement pool. If zero PIP payments were made on the case, the maximum top offer cap is \$6,000.

Diagnostics & Special Procedures

- **EMC or Range of Motion Testing:** Maximum top offer is capped at \$350 – \$400 if no PIP payments were made. If PIP paid between \$200 – \$500, cap the top offer at \$100. If PIP paid under \$200, the top offer cap is \$150.
- **MRI Facilities (Standard Scans):** If zero PIP coverage was applied, the maximum top offer is \$800 – \$1,000 per scan view. If PIP paid the standard fee schedule (\$930 – \$997), your top negotiation offer cannot exceed \$225 per scan view. For Brain MRIs without PIP intervention, the absolute max offer is \$1,100.
- **Interventional Pain Management:** If zero PIP coverage was applied, cap Epidural Steroid Injections, Facet Joint Injections, and Medial Branch Blocks at a flat \$1,500 per procedural date (even if multiple blocks or views were performed on that single day).



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Joint/Bursa drainage procedures are capped at \$600. Platelet-Rich Plasma (PRP) therapies are limited to \$900 per date.

- **Orthopedic MD Specialist Visits:** Initial orthopedic evaluations are capped at \$300; subsequent clinical follow-up visits are capped at \$150 (contingent on zero PIP deployment).
- **Surgical Centers & Advanced TBI Procedures:** Injection procedures performed at a surgical center are capped at \$800. Radiofrequency Ablation / Rhizotomy (RFA) holds a firm cap of \$11,000. For Traumatic Brain Injury testing protocols, cap functional cortical mapping at \$900 and basic vestibular profiles at \$400.

5. Advanced Cross-Referencing of Disparate Case Data

A. Prior Injury History vs. Recent Claim (The Structural Aggravation Standard)

When a baseline background check reveals a historical injury to the same anatomical region, defense adjusters will automatically try to attribute 100% of the current treatment to pre-existing degenerative disk disease (DDD) or past trauma. Case Managers must explicitly dismantle this argument by executing a comprehensive diagnostic cross-examination:

- **Secure and Extract Baseline Diagnostics:** Subpoena or request all medical records, chiropractic notes, and specifically the raw imaging data and reports from the historical baseline. You cannot rely on summary mentions in subsequent records; you must possess the actual pre-accident radiological findings.
- **Execute a Line-by-Line MRI Matrix Comparison:** Map the pre-accident radiological report directly against the new post-accident MRI findings. Look for definitive structural mutations. For example, if the baseline MRI notes a 2mm stable disc bulge at L4-L5 with no neural encroachment, and the post-collision MRI reveals a 5mm focal disc herniation with severe left-sided nerve root impingement, this establishes objective, measurable structural degradation.
- **Frame the Legal-Medical Narrative:** In the demand body, avoid generic soft-tissue language. Use specific terms like 'acute, traumatic structural advancement' and 'bony and neurological aggravation of a vulnerable spine.' Weaponize the Eggshell Plaintiff Doctrine—the tortfeasor takes the victim as they find them, and the defendant is entirely liable for the clinical acceleration of a pre-existing condition from an asymptomatic state to a chronic symptomatic state.



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B. Property Damage (PD) Severity vs. Treatment Intensity (Overcoming the MIST Defense)

Insurance companies routinely utilize automated software to flag claims with vehicle property damage estimates under \$500 as MIST (Minimum Impact Soft Tissue) files. These cases are structurally restricted by adjusters who argue that a low-velocity impact cannot generate sufficient kinetic energy to cause severe spinal pathology. Case Managers must aggressively counter this defense by compiling internal vehicular and biomechanical evidence:

- **Audit the Interior Cabin for Structural Damage:** Low external bumper damage frequently masks catastrophic energy absorption inside the vehicle cabin. Order a deep review of all property damage photos, salvage reports, and repair supplements. Search for and document clues such as bent seat frames or tracks, cracked steering wheel plastics, knee-strike markings on the lower dash column, or deployed seatbelt pretensioner locks. Check repair supplements for hidden damage behind the plastic fascia (e.g., a crushed foam absorber or bent rear impact bar).
- **Document and Reconstruct Occupant Position Mechanics:** The vector of force combined with the occupant's precise body positioning dictates injury severity far more than external sheet metal damage. Detail if the client's head was turned fully to the left or right at the split second of impact, which introduces complex twisting forces into the spinal ligaments and facet joints, severely heightening structural vulnerability at low speeds. Note if the client was forcefully bracing down on the brake pedal, which bypasses normal joint damping and transfers kinetic force directly up through the lower extremities into the lumbar spine, resulting in traumatic joint shear.

6. Firm Case Costs & Expense Calendars

Every active pre-litigation and litigation cost must be logged inside Casepeer and added directly to your Net Sheet.

Pre-Litigation Investigation Fees (Add123 Platform)

Driver's License Background Search: \$3.42 (Historical files prior to 3/14/2024: \$1.89)

VIN / Vehicle History Search: \$3.42 (Historical files prior to 3/14/2024: \$1.74)

Tag and License Plate Registration Search: \$1.74

State Vehicle Insurance Coverage Query: \$3.00



Certified Legal Mailing Rates

Regular First Class Mail: \$0.69 (Historical Jan 2024 scale: \$0.64)

Priority Mail: \$8.50

Certified Mail: \$8.69

Priority Signature Confirmation (Client Settlement Checks): \$12.20 (Historical Jan 2024 scale: \$12.00)

Core Litigation Cost Categories

Filing and Summons: Initial civil filing fees and complaint routing (\$414.00 to \$415.01) plus summons processing (\$10.35 per issued summons).

Process Server: Field charges for personal service of process on defendants.

Transcripts & Interpretation: Deposition court reporter transcript rates and certified translators used during adverse party depositions.

Medical-Legal Costs: Defense Compulsory Medical Exam (CME) videographer sessions and mandatory 'no-show' fees if a client misses a defense appointment.

Resolution Fees: Mediator/arbitrator retainer splits, translation costs for active mediation, and clinical expert witness case evaluations.

7. Document Assembly, Delivery & Routing Protocols

Step 1: Execute Document Assembly & Approvals

Once all outstanding items are gathered, initialize your sections on the Demand Checklist, sign and date the top header, and place the fully sorted chronological PDF into Sharepoint. Call the insurance provider's toll-free line to explicitly confirm the active handling bodily injury adjuster's name, phone, and direct email address before generating your template. Email your compiled demand package, signed checklist, and drafted letter to the handling attorney for review and formal sign-off (CC the pre-lit team email and your team leader). For UM demands, execute the exact same process from the Plaintiff Insurance module.

Step 2: Establish Expiration Timelines

- **Pre-Suit Status Files:** The response window on your demand letter is strictly 14 days.



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- **Active Litigation Status Files:** Leave the response deadline blank and defer entirely to the instructions of your approving attorney.

Step 3: Mailing Execution Protocols

Demands are submitted strictly via email if available. If no email option is open for that carrier, send a request to reception via email with the exact subject line: OUTGOING MAIL REQUEST – Demand packet – [Client Name] – [DOL] to burn a secure document CD. Always verify transmission by checking for a scanned envelope copy inside the Sharepoint Proof of Mailing folder.

Step 4: Casepeer Data Logging

Log the outgoing demand under Casepeer's Settlement section. Input the insurance category, policy boundaries, exact date sent, and formal expiration timeline. Set the fee logic parameters to 33.33% if no defense Answer was filed, or scale it to 40.00% if an active litigation Answer is documented.

Step 5: System Routing & Post-Demand Tracking

- **For Active Litigation Status Files:** DO NOT alter the Casepeer status code. Create follow-up tasks to check on adjuster receipt within 3 days and track the response timeline precisely on your calendar. If an actual offer is received, log the numbers under CP's Settlement tab, notify the approving attorney via email, and keep the case status exactly as it is.
- **For Pre-Suit Status Files:** Transition the global Casepeer file status to Demanded. Re-assign follow-up tasks to yourself at 3 days (to confirm receipt) and on the exact date of expiration. Once an offer is secured, log the numbers in Casepeer and change the file status code to Settlement Negotiations.